

All figures sourced from Government of Alberta open data, Statistics Canada, federal statute, UN treaty, and peer-reviewed research.

Building Up: A Blueprint for the Alberta Accessibility and Inclusion Act

Part One — The Architecture Alberta Built and the Lifecycle Failures It Produced

Section 1.10 — Aging, Acquired Disability, and End-of-Life as a Lifecycle Dimension

Sections 1.7, 1.8, and 1.9 documented mental health and addiction, rural-and-remote-and-northern Alberta, and the engineered disability-and-poverty intersection as substantive lifecycle dimensions. Section 1.10 documents the substantive lifecycle dimension that completes the frame established in Section 1.2: the substantive aging dimension, the substantive acquired-disability-through-aging dimension, and the substantive end-of-life-care dimension. The lifecycle frame must engage the complete substantive human-lifecycle trajectory; the substantive disability-policy architecture must engage the substantive operational reality that persons with disabilities age, that aging produces acquired disability for substantial portions of the population, and that end-of-life care for persons with disabilities operates within substantive operational complications the disability-policy architecture must address.

The substantive lifecycle dimension is operationally significant. AISH eligibility ends at age sixty-five. The substantive AISH-to-Old-Age-Security transition produces a substantive policy boundary at age sixty-five. The substantive Alberta record on long-term care and continuing care infrastructure operates within structural conditions documented to be substantively inadequate. The substantive intersection of long-term-care-infrastructure inadequacy with the engineered structural-poverty pattern documented in Section 1.9 produces the substantive forced-spousal-separation pattern that operates against senior couples whose substantive operational reality requires institutional care — a pattern the substantive academic-and-journalistic record documents and that the substantive frontline-care-work record has witnessed for decades. The substantive dementia-care infrastructure operates within structural capacity-and-quality conditions that operate substantively below the substantive needs the population presents. The substantive supported-decision-making framework for older adults with disabilities operates within the substantive Alberta *Adult Guardianship and Trusteeship Act* infrastructure with substantive operational complications the international human rights framework requires the disability-policy architecture to address.

The section develops the substantive content in nine components. The first establishes the substantive AISH-to-Old-Age-Security transition at age sixty-five and the operational implications. The second addresses the substantive distinction between aging-with-disability and acquired-disability-through-aging. The third documents the substantive Alberta record on long-term care and continuing care infrastructure. The fourth documents the substantive forced-spousal-separation pattern as a distinct substantive structural finding. The fifth addresses the substantive dementia-care infrastructure. The sixth addresses the substantive supported-decision-making framework. The seventh addresses the rural, remote, and northern Alberta aging dimension that compounds with the operational realities documented in Section 1.8. The eighth addresses end-of-life-care availability for persons with disabilities while acknowledging the substantive medical-assistance-in-dying dimension as requiring its own substantive analytical engagement that the blueprint does not undertake. The ninth synthesizes what the section establishes for

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the substantive case the blueprint develops.

The substantive content is grounded in primary-source documentation: the Alberta *Continuing Care Act* (effective April 1, 2024) and the Alberta Health Services continuing care infrastructure; the provincial-government data on long-term-care capacity and wait times; the substantive academic record on spousal separation in long-term care including the JBI systematic-review research, the Queen's University doctoral research (Madison Robertson, "Till Death Do Us Part: Spousal Separation in Long-Term Care"), and the broader peer-reviewed literature on loneliness, isolation, dementia, and mortality risk; the CBC News national reporting documenting the operational pattern across multiple Canadian jurisdictions including Alberta; the substantive frontline-care-work documentary record; the federal Old Age Security and Guaranteed Income Supplement architecture; and the Alberta *Adult Guardianship and Trusteeship Act* framework.

The AISH-to-Old-Age-Security Transition at Age Sixty-Five

The substantive AISH eligibility framework documented in Section 1.4 establishes age sixty-five as a substantive policy boundary. AISH eligibility ends at age sixty-five; AISH recipients are transitioned to the federal Old Age Security pension and the Guaranteed Income Supplement, with the provincial-government Alberta Seniors Benefit operating as a supplementary income-support layer. Source: <https://www.alberta.ca/aish-eligibility>; <https://www.alberta.ca/aish>.

The substantive operational consequences of the AISH-to-OAS transition include three substantive dimensions.

First, the substantive benefit-level architecture changes substantively. The federal OAS pension and the GIS, combined with the Alberta Seniors Benefit, operate at substantive aggregate levels that may operate above or below the AISH benefit-level for any individual recipient depending on the substantive operational details of their pre-sixty-five economic profile and the substantive interaction with other federal-and-provincial benefit programs. The substantive operational complications include substantive operational coordination gaps between federal and provincial benefit programs, substantive operational changes in benefit-eligibility-criteria during the transition period, and substantive operational changes in benefit-level-calibration that may produce real-dollar reductions for some senior-disability recipients.

Second, the substantive disability-related-supports architecture changes substantively. The AISH health-benefits architecture (prescription medications, some dental services, optical services, some Alberta Aids to Daily Living program coverage, some essential diabetic supplies) operates within the AISH framework. The senior-benefit architecture operates within the provincial-government Alberta Seniors Benefit framework, the federal Old Age Security and Guaranteed Income Supplement architecture, and the provincial-government Coverage for Seniors program. The substantive operational complications include changes in disability-related-supports coverage during the transition, gaps in continuity-of-services produced by the architectural change, and operational complications for senior-disability recipients whose substantive operational reality requires disability-related-supports operating within one framework but not the other.

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Third, the substantive disability-related-engagement infrastructure changes substantively. The AISH program administration through provincial-government regional offices operates substantively distinctly from the senior-benefit program administration through federal Service Canada offices and provincial-government Alberta Seniors Benefit administration. The substantive operational complications include structural-engagement complications produced by the architectural change, complications for senior-disability recipients who must navigate substantively distinct administrative systems, and operational gaps in disability-related-advocacy infrastructure that operate within the working-age-disability framework but not within the senior-disability framework.

The substantive AISH-to-OAS transition at age sixty-five thus operates as a substantive policy boundary that produces operational complications across three substantive dimensions. The substantive operational reality is that the disability-policy architecture treats age sixty-five as a substantive lifecycle break, with substantive consequences for the substantive continuity of disability-related supports across the substantive operational reality of disability that does not break at age sixty-five.

Aging-with-Disability and Acquired-Disability-Through-Aging

The substantive aging-and-disability dimension operates across two substantive populations with operationally distinct profiles.

Aging-with-disability addresses persons whose substantive disability profiles emerged earlier in life and who age into senior status. The substantive operational reality is that medical-and-rehabilitative advances over the past several decades have substantively extended the life-expectancy of persons with disabilities including persons with developmental disabilities, persons with cognitive disabilities, persons with physical disabilities arising from accidents or congenital conditions, persons with chronic-illness profiles, and persons with mental-health-disability profiles. The substantive Alberta record on this population includes the substantive PDD program documented in Section 1.4 (the population of adults with developmental disabilities receiving PDD services who age into senior status), the substantive AISH-recipient population documented in Sections 1.4, 1.7, and 1.9 (substantial proportions of whom are in older age cohorts and approach the AISH-to-OAS transition), and the substantive WCB-recipient population documented in Section 1.6.

The substantive operational complications for the aging-with-disability population include: complications produced by the AISH-to-OAS transition; complications produced by the substantive transition from PDD-administered services to senior-services-administered services for persons with developmental disabilities; complications produced by the substantive interaction between the underlying disability profile and the aging-related-conditions that emerge alongside the underlying disability; complications for the caregiver dimension documented in Section 1.5 where caregivers themselves age into senior status alongside the persons-with-disabilities they care for; and complications for the housing-and-community-supports architecture that the aging-with-disability population requires.

Acquired-disability-through-aging addresses persons whose substantive disability profiles emerge through the aging process itself. The substantive operational reality includes persons who develop dementia in later life; persons who experience strokes, heart attacks, or other acute-medical events that

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produce sustained disability profiles; persons who develop progressive conditions including Parkinson's disease, multiple sclerosis (which often presents earlier but progresses through aging), arthritis-and-mobility-related conditions, vision-and-hearing impairments, and chronic-illness profiles; and persons whose accumulated substantive aging-related changes produce sustained disability profiles even where no single discrete medical event marks the onset.

The substantive operational complications for the acquired-disability-through-aging population include: complications produced by the absence of pre-existing engagement with the disability-policy architecture (these persons did not engage the AISH/PDD/FSCD/WCB architecture during working-age years and may lack the operational knowledge and infrastructure to navigate disability-related supports); complications produced by the substantive interaction between the acquired disability profile and the substantive senior-benefit architecture; complications produced by the substantive housing-and-community-supports architecture inadequacy documented in the next subsection; and complications produced by the substantive medical-and-clinical-engagement patterns that operate distinctly for senior populations relative to working-age populations.

The substantive operational distinction between aging-with-disability and acquired-disability-through-aging is operationally significant for the substantive policy responses developed in Part Three. The substantive Section 3.3 Area 6 health-area standard, the Section 3.3 Area 5 services-and-programs standard, and the Section 3.7 implementation-timeline-and-funding provisions must address both substantive populations within the substantive standards architecture rather than treating either as the default substantive case.

Long-Term Care and Continuing Care Infrastructure

The substantive Alberta record on long-term care and continuing care infrastructure operates within structural conditions documented to be substantively inadequate at the provincial level. The Alberta *Continuing Care Act* (effective April 1, 2024) operationalizes a substantive provincial-government framework that updates the terminology and the substantive operational structure of the provincial continuing-care system, renaming long-term care facilities as "Continuing Care Home Type A" and designated supportive living facilities as "Continuing Care Home Type B," operating within the Alberta Health Services Continuing Care Access Centre system that requires Case Manager engagement for service access. Source: <https://www.albertahealthservices.ca/cc/Page15536.aspx>; <https://www.albertahealthservices.ca/cc/Page15491.aspx>.

The substantive provincial-government investment in continuing care under Budget 2024 was one-and-seven-tenths billion dollars, including a one-billion-dollar three-year commitment "to transform continuing care in Alberta." Source: <https://www.bigwestcountry.ca/2024/03/15/albertas-government-announces-funding-for-improved-access-to-healthcare/>. The substantive operational scale of the documented capacity gap operates as follows.

The alternate-level-of-care patient population. Approximately one thousand five hundred Albertans operate as "alternate level of care" (ALC) patients in acute-care hospitals at any given time, having completed their required hospital care but waiting in hospital beds for transfer to continuing-care facilities. The ALC-patient population represents approximately eighteen percent of all acute-care spaces in the

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province. Source: <https://www.bigwestcountry.ca/2024/03/15/albertas-government-announces-funding-for-improved-access-to-healthcare/>. The substantive operational implication is that the substantive structural-capacity gap in the continuing-care infrastructure operates with substantial operational consequences for the broader healthcare system, including substantive structural-pressure on emergency-department capacity, surgery wait-times, and the substantive operational efficiency of the hospital-and-acute-care infrastructure.

The continuing-care wait-times. Wait times for continuing-care placement in Alberta operate at substantial scale, ranging from six months to two years depending on the substantive level of care required, the substantive geographic region, and the substantive specific facility. Source: <https://www.seniorcareaccess.com/article/A%20Guide%20to%20Long-Term%20Care%20in%20Canadian%20Provinces>. The Health Quality Alberta tracking infrastructure documents the percentage of residents placed into continuing-care homes within thirty days of assessment as a substantive operational performance indicator. Source: <https://focus.hqa.ca/charts/placement-into-continuing-care-facility-living/>.

The provincial-government acknowledged capacity need. The substantive provincial-government's own acknowledged capacity need is ten thousand new spaces over three years — a scale substantively above the documented one-thousand-fifty-additional-spaces target the provincial government had committed to opening by early 2025 through temporary expansion of capacity in existing facilities. Source: <https://www.bigwestcountry.ca/2024/03/15/albertas-government-announces-funding-for-improved-access-to-healthcare/>.

The broader healthcare wait-times context. The Fraser Institute's *Waiting Your Turn: Wait Times for Health Care in Canada, 2024* documents the substantive Alberta median wait-time for orthopedic surgeries as sixty-six-and-eight-tenths weeks, and the median wait across twelve medical specialties as twenty-nine-and-five-tenths weeks. Source: <https://www.fraserinstitute.org/categories/health-care-wait-times>. The substantive operational implication for the senior-disability population is that the medical-and-surgical-and-specialist services that the population requires operate within wait-times substantively above the substantive operational reality the population's substantive needs require.

The Forced-Spousal-Separation Pattern

The substantive intersection of the long-term-care-infrastructure inadequacy documented in the prior subsection with the engineered structural-poverty pattern documented in Section 1.9 produces the substantive forced-spousal-separation pattern. The pattern is documented at the national level in Canadian journalism, in academic research, in disability-and-aging advocacy work, and in the substantive frontline-care-work documentary record that long-term-care workers across decades have witnessed and named. The pattern operates across three substantive operational layers, produces a documented health-and-stability cascade that the peer-reviewed medical literature anchors, and generates a structural-feedback loop that compounds the substantive case the blueprint develops.

The bed-availability layer. The substantive operational reality is that the substantive long-term-care infrastructure does not adequately accommodate spousal pairs. The substantive operational

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complications include facility-specialization patterns that distribute substantive levels of care across distinct facilities (with the operational consequence that one spouse may require a substantive level of care — memory care, complex-medical care, palliative care — operating in a facility that does not also accommodate the other spouse); bed-availability constraints that produce the operational reality that securing two adjacent or available spaces in the same facility for a couple requiring placement simultaneously is operationally improbable given the system-level capacity gaps documented in the prior subsection; room-availability patterns where many continuing-care facilities operationally provide single-occupancy rooms with limited or no provision for couples to share accommodation; and substantive coordination gaps between the substantive different categorical-program funding-streams that operate across the continuing-care system. Source: <https://www.todays-woman.net/2025/health-personal-fitness/a-ging-and-seniors/separating-couples-in-long-term-care-heartbreaking/>.

The substantive Alberta-specific case documented in CBC News national reporting (June 2019) establishes the operational reality. Martha and Willard Farnell of Calgary, Alberta, married for sixty-six years, were forced to live apart following Willard's dementia diagnosis and admission to a long-term-care facility approximately twenty minutes' drive from Martha's residence. The substantive operational reality for the Farnell family is that the Alberta provincial-government continuing-care system did not provide a substantive operational mechanism through which the spousal-pair could continue to share accommodation when one spouse required institutional care that the other spouse did not require. Source: <https://www.cbc.ca/news/health/national-long-term-care-couples-housing-1.5159102>.

The financial-structure layer. The substantive operational reality is that the substantive long-term-care fee-determination architecture operates on the basis of couple's-combined-income calculations, producing substantive operational complications for the spouse who remains in the community. The substantive operational pattern is that long-term-care facility costs may operate at substantial proportions of the couple's combined income, with documented cases including facilities requesting sixty percent of one spouse's pension income and forty percent of the other's to permit institutional care. Source: <https://www.cbc.ca/news/canada/new-brunswick/nursing-home-costs-leave-seniors-pondering-divorce-1.3171888>.

The substantive operational consequence is that some senior couples face the substantive operational choice between accepting the substantive financial-precarity that the institutional-care-fee structure imposes or pursuing legal divorce as an asset-protection strategy. The substantive documented cases include the Kenneth Campbell case (divorced wife so his assets would not be included in nursing-home fee calculation, continuing to visit her daily but no longer financially obligated), the Jerry Clarke case (considering divorce after fifty-two years to deal with seven-hundred-dollar monthly increase in wife's nursing-home cost), the Judy MacKenzie case (announced 2009 plan to separate from husband of forty-five years over seven-hundred-dollar monthly contribution requirement), and historical cases including Elizabeth Martin (declared on the New Brunswick legislature steps in 2003 that she was prepared to file for divorce because she feared the province would take her assets). Source: <https://www.cbc.ca/news/canada/new-brunswick/nursing-home-costs-leave-seniors-pondering-divorce-1.3171888>.

The federal government provides a substantive partial mitigation through the Old Age Security Act *involuntary separation* provision, which permits both spouses to be treated as single persons for OAS and GIS calculation purposes when one spouse is in a long-term-care facility (or where both spouses are in long-term-care facilities, regardless of whether they share a room). Source:

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<https://www.familycaregiversbc.ca/caregiver-learning-center/read-resources/involuntary-separation>. The substantive operational consequence of the involuntary-separation provision is that the OAS-and-GIS benefit calculation may operate at higher single-individual levels than at couple levels for the substantive duration of the long-term-care placement. The substantive operational limitation is that the involuntary-separation provision operates as a partial financial mitigation that does not address the bed-availability layer (the spousal-pair remains structurally separated by the long-term-care infrastructure regardless of the benefit-calculation adjustment) and that does not always produce sufficient real-dollar protection to prevent the forced-divorce-as-asset-protection pathway.

The family-driven separation layer. The substantive operational pathway through which the bed-availability and financial-structure layers produce documented separations is frequently the family-driven pathway. Adult children, extended family, and broader family-and-community networks operating in the substantive operational role of supporting senior couples in care transitions face the substantive structural reality that the long-term-care fee architecture exceeds the couple's combined income capacity, that the family cannot afford to subsidize the cost-of-care difference on top of their own household obligations (mortgages, dependent children, their own healthcare costs, the working-age financial pressures that operate across every population the prior sections of Part One have documented), and that the asset-protection-and-benefit-maximization-through-legal-separation pathway operates as the structural-financial-survival strategy available within the substantive policy architecture. The substantive operational consequence is that families step in and pursue legal separation or divorce on behalf of senior couples whose substantive operational reality is that the couple wishes to remain together but the substantive financial architecture leaves no other operational pathway. The substantive frontline-care-work documentary record across decades has witnessed this pattern operationally at the substantive scale the documented data only partially capture; long-term-care workers, geriatric-care providers, and frontline social-services staff have observed the family-driven separation pathway as a recurring operational pattern within the substantive Alberta long-term-care system.

The family-driven separation pathway operates with substantive operational distinctions from the spouse-initiated separation pathway. The senior couple does not, in the substantive operational reality, freely choose separation; the family acts as substantive operational decision-maker on behalf of the couple in response to the substantive financial-and-bed-availability constraints. The substantive operational consequences include substantive distress for the senior couple themselves, who experience the separation as imposed rather than chosen; substantive distress for the family-decision-makers, who experience the substantive moral-and-emotional weight of pursuing legal separation against their parents' or grandparents' substantive operational wishes in the structural-financial-survival context; and substantive structural-relationship complications across the multi-generational family system that produce sustained operational consequences beyond the immediate substantive senior-couple experience.

The documented health-and-stability cascade. The peer-reviewed medical literature documents that the operational consequences of forced-spousal-separation in long-term-care contexts include substantive measurable impacts on senior couples' health, cognitive function, mental health, and mortality risk. The substantive operational evidence includes: the documented finding that severe loneliness affects up to forty-two percent of long-term-care residents, a substantively higher prevalence than the ten percent prevalence in older adults living in the community (Source: <https://jbi.global/news/article/understanding-lo>

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neliness-and-depression-spousal-separation-within-long-term-care); the documented finding that loneliness is associated with the transition from no cognitive impairment to mild cognitive impairment, and that isolation is associated with the transition from mild to severe cognitive impairment (Source: <https://www.ncbi.nlm.nih.gov/pmc/articles/PMC12761437/>); the documented finding that loneliness is associated with elevated mortality risk regardless of age, sex, race, or ethnicity, with up to forty percent of the total effect on mortality operating as a "pure indirect effect" mediated through dementia probability (Source: <https://www.ncbi.nlm.nih.gov/pmc/articles/PMC11713336/>); the documented finding that spousal loss is associated with increased dementia risk and mortality, with the first year post-separation operating as a particularly high-risk period (Source: <https://www.bu.edu/sph/news/articles/2026/spousal-loss-linked-to-higher-risk-of-dementia-mortality-among-men-but-not-women/>); and the documented finding that probable dementia is consistently linked to over twofold increased mortality risk.

The substantive Queen's University doctoral research project by Madison Robertson, *Till Death Do Us Part: Spousal Separation in Long-Term Care*, examined the mental-health impact of spousal separation in long-term-care facilities through direct engagement with residents and their spouses, addressing the substantive research-gap that the prior literature on the topic primarily focused on the spouses living outside long-term-care rather than on the residents themselves. Source: <https://nursing.queensu.ca/stories/research/researching-mental-health-impact-imposed-separation-long-term-care>. The research was recognized at the national level through the SSHRC Storytellers Challenge, establishing the substantive academic significance of the topic.

The peer-reviewed literature on spouses living outside long-term-care after their partner moves into a care home documents the substantive concept of "ambiguous loss" — the substantive grief experience that operates when the partner is operationally absent (in care) but not deceased, producing sustained psychological complications including depression, grief, and loneliness for the at-home spouse alongside the institutionalized-spouse's substantive experience. Source: <https://pmc.ncbi.nlm.nih.gov/articles/PMC12340749/>.

The structural-feedback loop. The cumulative substantive finding is that the substantive financial-and-structural decision to pursue forced-spousal-separation produces the documented health-and-stability cascade, which operationally generates substantive subsequent increases in the cost of care — cognitive decline produces additional medical and care costs; mental-health deterioration produces additional treatment costs; physical-health deterioration produces additional hospitalization and acute-care costs; mortality risk produces additional end-of-life-care costs. The structural-financial decision that the substantive engineered structural-poverty pattern compels thus produces the substantive medical-and-care-cost increases that the substantive policy architecture is structurally responsible for absorbing through the public healthcare system. The substantive feedback loop operates against the long-term substantive fiscal interests of the policy architecture itself, in addition to operating against the substantive lifecycle-disability experience of senior couples and their families.

The substantive case the blueprint develops engages this pattern substantively. The substantive operational mechanism producing the forced-spousal-separation pattern is the substantive engineered structural-poverty pattern documented in Section 1.9, operating against the senior-disability population through the long-term-care fee-determination architecture, the bed-availability infrastructure, and the cohabitation-penalty pattern that the categorical-program architecture imposes across the lifecycle

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dimensions. The substantive Part Three Section 3.3 Area 5 (services and programs standard) and Section 3.3 Area 6 (health area standard) operationalize substantive policy responses including substantive bed-availability provisions for spousal pairs, substantive fee-determination architecture that does not produce financial-precarity for the at-home spouse, substantive supported-decision-making infrastructure that operates against the family-driven-separation pathway, and substantive integration with the international human rights framework Article 19 (living independently and being included in the community) and Article 23 (respect for home and the family) provisions.

Dementia Care Infrastructure

The substantive dementia-care infrastructure within the Alberta long-term-care system operates as a substantive sub-component of the broader continuing-care system documented in the prior subsections. The substantive operational reality is that dementia-care produces specific substantive operational requirements that the broader continuing-care infrastructure does not always adequately accommodate, including: specialized memory-care units operating with substantive design-and-operational features (secure environments, sensory-design features, structured-routine provisions, specialized-staffing-and-training requirements); substantive distinct facility designs for advanced-dementia-care that produce the substantive bed-availability complications documented in the prior subsection; specialized-care-team training requirements operating substantively above the broader continuing-care-team training; and substantive operational coordination requirements for dementia-care that operate distinctly from the broader continuing-care coordination.

The substantive provincial-government dementia-care infrastructure operates within the substantive provincial-government Continuing Care Act framework with substantive operational coordination through Alberta Health Services. The substantive operational complications include the documented continuing-care capacity gaps that compound substantively for dementia-care infrastructure (where the substantive specialization requirements produce additional operational constraints on the broader capacity gaps), the substantive provincial-government workforce-recruitment-and-retention gaps documented in Section 1.8 (which compound substantively for specialized memory-care staffing), and the substantive geographic-distribution gaps documented in Section 1.8 (which compound substantively for specialized memory-care infrastructure that requires specialized-design-and-staffing capacity not available in many rural-and-remote communities).

The substantive Alberta dementia-population trajectory operates within the substantive broader Canadian dementia-population trajectory, which the Alzheimer Society of Canada documents as a substantive population-and-demand growth trajectory operating substantively above the substantive infrastructure-and-capacity-development trajectory. The substantive operational implication is that the substantive structural-capacity gap in dementia-care infrastructure is documented to grow over the period and to compound substantively with the broader continuing-care capacity gaps documented in the prior subsections. The substantive operational consequence for the population the policy architecture is intended to serve is that the documented forced-spousal-separation pattern operates with particular operational force in dementia-care contexts (where the substantive specialization requirements produce additional bed-availability constraints), the documented health-and-stability cascade operates with

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particular operational force (where dementia-related cognitive-decline operates within the substantive isolation-and-loneliness mortality pattern), and the substantive family-decision-maker pathway operates with particular operational complexity (where the dementia-affected spouse may lack substantive decision-making capacity and where the at-home spouse and family operate within substantive emotional-and-practical complications that the broader continuing-care contexts may not present at the same operational scale).

Supported Decision-Making for Older Adults with Disabilities

The substantive Alberta supported-decision-making framework operates within the *Adult Guardianship and Trusteeship Act* (AGTA) infrastructure, which provides for substantive guardianship-and-trusteeship arrangements for adults whose capacity to make personal-and-financial decisions is operationally impaired. The substantive operational complications for older adults with disabilities include: substantive operational tensions between the substantive guardianship-and-substitute-decision-making framework and the substantive Article 12 (equal recognition before the law) provisions of the United Nations *Convention on the Rights of Persons with Disabilities* that operate in favour of substantive supported-decision-making rather than substitute-decision-making; substantive operational complications for older adults with dementia or other cognitive-impairment profiles whose substantive capacity for decision-making operates fluctuationally and across decision-domains; substantive operational complications for older adults whose substantive operational reality includes substantive family-system dynamics that produce substantive complications for the substantive operationalization of supported-decision-making; and substantive operational complications for older adults operating within the long-term-care infrastructure documented in the prior subsections, where the substantive institutional context may produce substantive structural pressures that operate against the substantive autonomy-and-supported-decision-making provisions.

The substantive Article 12 international human rights framework, as developed through the Committee on the Rights of Persons with Disabilities General Comment No. 1 (on equal recognition before the law), establishes the substantive policy direction toward supported-decision-making frameworks operating against substitute-decision-making frameworks. The substantive Alberta AGTA framework operates substantively within the substitute-decision-making model that the substantive Article 12 framework identifies as requiring substantive policy reform across signatory jurisdictions. The substantive Part Three Section 3.3 Area 6 (health area standard) develops the substantive supported-decision-making provisions that operationalize the substantive Article 12 framework within the substantive Alberta context.

The Rural, Remote, and Northern Alberta Aging Dimension

The substantive aging-and-disability dimension operates with particular operational force in rural, remote, and northern Alberta as documented in Section 1.8. The substantive operational reality is that the substantive proportion of the rural Alberta population aged sixty years or older is substantively higher than the substantive proportion in urban Alberta: sixteen-and-six-tenths percent in rural Alberta compared to thirteen-and-seven-tenths percent in the Census Metropolitan Areas (Source: Alberta's Rural

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Communities Report). The substantive operational implication is that the substantive aging-related-disability population operates substantively concentrated in the substantive geographic regions where the substantive long-term-care-infrastructure-capacity, substantive dementia-care-infrastructure-capacity, substantive workforce-recruitment-and-retention infrastructure, and substantive specialist-services availability all operate substantively below the urban Alberta levels.

The substantive operational complications for the rural-remote-northern aging-and-disability population include: substantive geographic-distance from continuing-care facilities that may require substantive relocation away from established communities and family-and-social networks (with the substantive consequence that the documented isolation-and-loneliness mortality pattern compounds substantively for the relocated population); substantive geographic-distance from specialist medical-and-clinical services that produces substantive operational complications for the substantive medical-and-clinical engagement that the aging-related-disability population requires; substantive continuing-care-bed-availability gaps that operate substantively below the urban availability levels; and substantive forced-spousal-separation patterns that compound with the substantive geographic-distance dimension to produce structurally-deeper separations than the urban-context forced-spousal-separation patterns alone produce.

The substantive Indigenous-disability dimension developed in Section 1.8 operates with particular operational force in the rural-remote-northern aging-and-disability context. The substantive operational complications include: substantive jurisdictional-fracture complications for Indigenous-elder populations operating within the substantive Treaty-territory and Métis-Settlements frameworks; substantive cultural-and-spiritual-care provisions that operate substantively below the substantive operational reality of the population's needs in many continuing-care facility contexts; and substantive Indigenous-elder population-specific operational complications that the campaign's separate Indigenous Disability Alberta evidentiary record documents in detail.

End-of-Life Care for Persons with Disabilities

The substantive end-of-life-care dimension operates as a substantive operational requirement within the broader healthcare-and-disability-policy architecture. The substantive operational complications for persons with disabilities include: substantive operational complications for the substantive integration of palliative-care services with disability-related-supports infrastructure; substantive operational complications for the substantive supported-decision-making framework documented in the prior subsection where the substantive end-of-life-care decision-making operates within substantive cognitive-and-emotional contexts that produce additional supported-decision-making complications; substantive operational complications for the substantive integration of family-and-community supports with the substantive medical-and-clinical infrastructure; and substantive operational complications for the substantive cultural-and-spiritual-care provisions that operate within the substantive end-of-life-care context.

The substantive operational reality is that the substantive end-of-life-care infrastructure in Alberta operates within the substantive Alberta Health Services framework with substantive operational coordination through hospice-and-palliative-care providers, the substantive continuing-care infrastructure documented in the prior subsections, and the substantive primary-care-and-specialist medical infrastructure. The

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substantive operational complications for persons with disabilities operating within this infrastructure include all the substantive operational complications documented across the prior subsections of Section 1.10 and the prior sections of Part One, compounding within the substantive end-of-life-care context.

The substantive medical-assistance-in-dying (MAID) dimension operates as a substantively contested element within the broader end-of-life-care infrastructure, raising substantive operational and ethical complications that the substantive disability-community has engaged with substantive depth and substantive internal-disagreement. The substantive operational complications include substantive complications for the substantive Convention-alignment questions that the MAID provisions raise, substantive operational complications for the substantive supported-decision-making framework when applied within MAID contexts, substantive operational complications for the substantive disability-poverty intersection documented in Section 1.9 where the substantive engineered structural-poverty produces substantive operational pressures that may operate within MAID decision-making contexts, and substantive operational complications for the substantive disability-affirming-practice framework that the substantive case the blueprint develops requires. The substantive MAID dimension requires its own substantive analytical engagement, which the blueprint does not undertake; the substantive call Section 3.3 Area 6 makes (that MAID raises complex Convention-alignment questions and is not addressed) operates as the substantive call Section 1.10 also makes. The substantive case the blueprint develops engages the broader end-of-life-care dimension within the substantive standards architecture without engaging the substantive MAID-specific provisions, leaving that substantive engagement to subsequent substantive analytical work that the substantive disability-community has undertaken and continues to undertake within the substantive complexity the topic requires.

What Section 1.10 Establishes

The substantive lifecycle dimension of aging, acquired disability, and end-of-life is now documented at the depth required for the substantive case the blueprint develops. The substantive content establishes seven substantive findings.

First, AISH eligibility ends at age sixty-five, producing a substantive policy boundary that operates with substantive operational consequences across three substantive dimensions (benefit-level architecture changes, disability-related-supports architecture changes, disability-related-engagement infrastructure changes). The substantive disability-policy architecture treats age sixty-five as a substantive lifecycle break, with substantive consequences for the substantive continuity of disability-related supports across the substantive operational reality of disability that does not break at age sixty-five.

Second, the substantive aging-and-disability dimension operates across two substantive populations with operationally distinct profiles (aging-with-disability and acquired-disability-through-aging) that the substantive standards architecture developed in Part Three must address within the substantive content rather than treating either as the default substantive case.

Third, the substantive Alberta long-term care and continuing care infrastructure operates within structural conditions documented to be substantively inadequate at the provincial level, with approximately one thousand five hundred Albertans operating as alternate-level-of-care patients in acute-care hospitals at

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any given time (approximately eighteen percent of acute-care spaces), wait-times for continuing-care placement ranging from six months to two years, and the substantive provincial-government's own acknowledged capacity need of ten thousand new spaces over three years operating substantively above the documented one-thousand-fifty-additional-spaces target.

Fourth, the substantive forced-spousal-separation pattern operates as a substantive structural finding that compounds the substantive engineered structural-poverty pattern documented in Section 1.9 with the substantive long-term-care-infrastructure inadequacy documented in this section. The pattern operates across three substantive operational layers (bed-availability, financial-structure, family-driven-separation), produces a documented health-and-stability cascade anchored in peer-reviewed medical literature (forty-two percent severe-loneliness prevalence in long-term-care residents versus ten percent in community-dwelling older adults; loneliness associated with cognitive-impairment transitions and elevated mortality risk; spousal loss associated with increased dementia and mortality risk; the "ambiguous loss" pattern documented for at-home spouses), and generates a structural-feedback loop in which the financial decision produces the deterioration that increases the substantive cost of care.

Fifth, the substantive dementia-care infrastructure operates within structural capacity-and-quality conditions that operate substantively below the substantive needs the population presents, with substantive operational complications including specialized memory-care unit capacity gaps, workforce-recruitment-and-retention complications that compound for specialized staffing, and substantive geographic-distribution gaps that compound the rural-remote-northern dimension documented in Section 1.8.

Sixth, the substantive supported-decision-making framework for older adults with disabilities operates within the *Alberta Adult Guardianship and Trusteeship Act* infrastructure, which operates substantively within the substitute-decision-making model that the substantive United Nations *Convention on the Rights of Persons with Disabilities* Article 12 framework identifies as requiring substantive policy reform across signatory jurisdictions.

Seventh, the substantive rural, remote, and northern Alberta aging dimension operates with particular operational force across the operational complications documented across the prior components, with substantive aging-population concentration (sixteen-and-six-tenths percent age sixty or older in rural Alberta versus thirteen-and-seven-tenths percent in urban Alberta) compounding with the substantive long-term-care-infrastructure-capacity gaps, dementia-care-infrastructure-capacity gaps, and Indigenous-disability dimension complications documented in Section 1.8 and elsewhere in Part One.

The substantive content of Section 1.10 completes the lifecycle frame established in Section 1.2. The substantive lifecycle dimension of aging, acquired disability, and end-of-life operates substantively across the substantive disability-policy architecture's operational reality, and the substantive case the blueprint develops in Parts Two and Three engages the substantive content at the depth the documented Alberta record requires. The substantive Section 3.3 Area 5 (services and programs), Section 3.3 Area 6 (health), Section 3.7 (implementation timeline and funding), and Section 3.8 (statutory review and coordination) provisions operate substantively against the documented patterns through the substantive standards architecture, the substantive structural-funding mechanisms, and the substantive accountability-and-coordination layer the blueprint specifies.

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The substantive forced-spousal-separation finding operates as the most substantively dispositive element of Section 1.10's substantive content. The pattern is not produced by individual-couple circumstances, by family-decision-making preferences, or by senior-couple choices about where to live; it is produced by the substantive policy architecture's structural-financial design, the substantive long-term-care infrastructure's structural-capacity gaps, and the substantive engineered structural-poverty pattern documented in Section 1.9 operating with particular operational force against the senior-disability population. The substantive frontline-care-work documentary record across decades has witnessed this pattern operationally; the substantive academic-and-journalistic record has documented it analytically; the substantive case the blueprint develops engages it substantively at the standards-architecture level the international human rights framework requires.

The lifecycle frame is now complete. Sections 1.1 through 1.10 have documented the substantive disability-policy architecture Alberta has built and the substantive lifecycle failures the architecture produces across the complete substantive human-lifecycle trajectory. Part Two engages the substantive comparator record (federal Accessible Canada Act, the five comparator provincial accessibility statutes, the United Nations *Convention on the Rights of Persons with Disabilities*, and the substantive Alberta-as-outlier finding), and Part Three develops the substantive *Alberta Accessibility and Inclusion Act* legislative blueprint that operates substantively against the documented patterns through the substantive standards architecture, the substantive consultation and Indigenous-rights-integration framework, and the substantive structural-funding and accountability-and-coordination provisions.